

Is Nursing Safe From AI? The 2026 Degree Risk Score

The free sampler — the score by track, the six factors in brief, and the honest reasons to think twice. The complete profile goes deeper.

2.8

AI EXPOSURE · BEDSIDE RN (HOSPITAL, ACUTE CARE)
where 10 is most at risk

Re-scored every six months. We publish where we might be wrong.

Quick answer: Nursing scores **2.8 out of 10** for AI exposure — one of the lowest of the thirty careers we track, where 10 is most at risk. Bedside nursing is protected by physical presence, patient trust and licensure. But desk-based nursing roles like telehealth triage and case management score **5.4**, nearly double. **The protection lives at the bedside, not in the qualification.**

The short answer for parents: yes — nursing is one of the strongest degree bets we score. Protected by hands, trust and law, with an entry route that is legally guaranteed at a time when most professions are losing theirs. The catch is that the same degree can lead to work scoring 2.4 or 5.4, and that divergence usually shows up years later.

Nursing AI risk score: the full table

Every career in this index is scored 1–10, where **10 is most exposed** to AI. Same six factors, same weights, applied identically to a paramedic and a paralegal.

NURSING TRACK	2023	2025	FALL 2026	3-YR MOVE	BAND
Specialist clinical (ICU, ER, perioperative)	1.9	2.1	2.4	+0.5	Low
Community / home health nursing	2.2	2.4	2.7	+0.5	Low
Bedside RN (hospital, acute care)	2.1	2.4	2.8	+0.7	Low
Nurse practitioner / advanced practice	2.6	2.9	3.2	+0.6	Low
Telehealth triage nurse	4.1	4.8	5.2	+1.1	Moderate
Utilization review / case management	4.3	5.0	5.4	+1.1	Moderate

2023 and 2025 figures are reconstructed using current methodology, not archived from past editions.

How that compares: the median career in this edition scores around 5.5. Entry-level software development scores **8.1**. Graphic design production scores **8.4**. Physical therapy scores **2.5**.

Patient-facing medicine scores 2.9.

Every hands-on nursing track sits between 2.4 and 3.2. Every desk-based track sits above 5 — and the desk tracks are moving twice as fast.

Will AI replace nurses?

No, and not for the reason most people assume.

AI is already doing a meaningful share of nursing work — but it is the documentation share, not the care. Ambient documentation systems that listen to a nurse-patient interaction and generate the structured record are already deployed in hospitals. Care-plan drafting, shift handover summaries and protocol lookup are all being automated.

What is not automating is physical assessment, hands-on care, and recognizing that a patient is deteriorating before the monitor says so.

AI IS TAKING	IT CAN'T TOUCH
Charting and flowsheet entry	Physical assessment
Shift handover summaries	Spotting deterioration before the monitor does
Care-plan drafting	Hands-on care
Protocol and guideline lookup	Being the human in the room at a frightening moment
Documentation burden generally	Clinical judgment on an atypical presentation

AI goes after the routine, repeatable layer of any job first. That is true everywhere. What differs is whether that layer was the part people valued — and in nursing the automatable layer is the paperwork. Nobody became a nurse to chart.

For most working nurses this is good news. Documentation burden is among the most-cited reasons people leave the profession.

Why does nursing score so low? The six factors

FACTOR	RATING	EFFECT ON RISK
How much of this job can AI already do?	5.5	↑ moderate exposure
How hard will it be to land that first job?	2.0	↓ strong protection
Does it have to be done in person, with your hands?	9.0	↓ strong protection
Does someone need a human they can trust and hold responsible?	9.0	↓ strong protection
Does the law require a licensed human?	9.0	↓ strong protection
How often does the job hit genuinely new, high-stakes situations?	8.0	↓ good protection

Five of the six run in nursing's favor. The full profile explains the reasoning behind every rating and shows the arithmetic that produces 2.8.

ONE FACTOR, WORKED THROUGH IN FULL

So you can see what the analysis actually looks like.

How hard will it be to land that first job? — rated 2.0

This is the factor most careers advice ignores, and the one where nursing quietly outperforms almost everything else we score.

Across most professions, AI is absorbing the *junior* tasks first — document review, boilerplate code, first-draft analysis, grunt research. Those tasks were not just work; they were the apprenticeship. When they disappear, the profession stays healthy while the route into it narrows. Entry-level software developers rate 9.0 on this factor. Junior lawyers rate 8.5.

Nursing is nearly immune, for a structural reason: **clinical hours are mandated by licensure.** A nursing student cannot qualify without supervised real-world practice. That requirement is written into law, so it cannot quietly erode the way a market-driven on-ramp can. **An employer can decide to hire fewer graduates. No employer can decide a nurse doesn't need clinical hours.**

Add persistent shortages, and a new graduate enters one of the more welcoming job markets in this index. Rated 2.0 rather than 1.0 only because competition for the best placements and first posts is real, and rising.

Which nursing jobs are safest from AI?

Ranked by exposure, safest first:

1. **Specialist clinical** — 2.4. ICU, emergency, perioperative. Continuous novelty, time pressure, life-critical judgment, maximum physical involvement. 2. **Community and home health** — 2.7. Hands-on care in uncontrolled environments, often as the only clinician present. 3. **Bedside RN** — 2.8. The reference point for the profession. 4. **Nurse practitioner** — 3.2. Strongest regulatory position in nursing, but diagnostic reasoning is more automatable than physical care and more of the work is screen-based. 5. **Telehealth triage** — 5.2. Protocol-driven assessment without physical examination. 6. **Utilization review** — 5.4. Applying coverage criteria to documented facts, close to a definitional automation target.

The pattern holds across every profession we score: **move the same work behind a screen and exposure roughly doubles**. Teaching goes from 3.6 in a classroom to 6.8 online. Medicine goes from 2.9 with patients to 4.9 reading scans.

Is nursing a good career in 2026?

The AI question and the career question are different, and this index only answers the first.

Nursing scores well on exposure. It also has genuine difficulties that have nothing to do with technology: staffing pressure, shift work over decades, physical toll, and turnover concentrated at the start — 22.7% of newly hired RNs leave within their first year, and 56.8% of all departing staff had under two years of service.

At the same time, most nurses say they would choose the profession again, and the projected shortfall exceeds 250,000 registered nurses by 2028. But only 45% would choose the same specialty — which tells you where the real decision sits.

A low AI score does not mean an easy job. The full profile covers the downsides, the satisfaction data, and who this work actually suits, because those questions matter more to a real decision than the score does.

Common questions

Is a nursing degree worth it with AI? On exposure alone, yes — nursing is among the best-protected careers we score, and its entry route is legally guaranteed. The more useful question is which nursing track a graduate ends up in, since the same degree spans 2.4 to 5.4.

Will AI take over nursing documentation? Substantially, and it already is. Ambient documentation is deployed in hospitals now. Most nurses regard this as an improvement rather than a threat, since documentation burden is a leading reason people leave.

Is telehealth nursing at risk from AI? More than bedside nursing. Telehealth triage scores 5.2 against bedside's 2.8, because protocol-driven assessment without physical examination is much closer to what AI does well.

Should my child do nursing or medicine? Both score well — nursing at 2.8, patient-facing medicine at 2.9. The differences that matter are training length (2–4 years versus 10+ to independence), cost, and the nature of the work.

Is nursing safer than software engineering? On our scoring, considerably. Bedside nursing scores 2.8; entry-level software development scores 8.1. That surprises most people, and it is the most consistent finding in this index.

Related profiles

- [Medicine](#) — 2.9 patient-facing, 4.9 diagnostic specialties
 - [Physical therapy and allied health](#) — 2.5, the lowest score in this edition
 - [Psychology](#) — 3.5 licensed clinical, 7.1 with the degree alone
 - [Teaching](#) — 3.6 classroom, 6.8 online
 - [Computer science](#) — 8.1 entry-level, 5.4 senior. Free to read in full.
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What's in the full nursing profile

This sampler tells you where nursing stands. The full profile tells you what to do about it.

	SAMPLER	FULL PROFILE
Verdict, all sub-track scores, 3-year trend	✓	✓
Six-factor ratings	✓	✓
Reasoning behind every factor rating	one example	✓
How durable each protection is — where AI is already pressing		✓
The honest downsides — attrition, staffing, shift work		✓
What's genuinely good about it — satisfaction data, what nurses say		✓
Who this work suits, and who it doesn't		✓
The AI-native advantage — how to prepare, and why this cohort has an opening		✓
Routes in — BSN, ADN, accelerated, bridge, advanced practice		✓
Where the degree leads later — and which exits raise exposure		✓
Program evaluation checklist		✓
Questions to ask an admissions office — twelve, plus red flags		✓
Sourced further reading, including the strongest case against our score		✓
Discussion questions for parent and student		✓
A short version written directly to the student		✓
Technical scoring appendix		✓

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Each full profile includes everything in the table above, plus a short version written directly to the student and the technical scoring appendix. Spring 2027 updates of whatever you buy are included.

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[Read a complete profile free →](#) We publish one profile in full, openly, so you can judge the depth before buying anything. Computer science is the one to read — it's the most surprising result in the index.

Pivotum scores thirty careers against AI exposure using a fixed, published methodology, re-scored every six months. Scores measure exposure to what AI can already do — not how much any particular employer has deployed. We publish where we might be wrong.